



Fiona Stanley Fremantle Hospitals Group

OFFICIAL

PHARMACY DOCUMENT

Policy

Prescribing for Mental Health Patients

Reference #: FSFHG-MENH-POL-0007

Scope

Site	Service/Department/Unit	Disciplines
Fiona Stanley Hospital Fremantle Hospital	Mental Health	Medical, Nurse Practitioner and Pharmacy

1. Introduction

This policy aims to facilitate contemporary evidence-based practice medicines prescribing for Fiona Stanley Hospital and Fremantle Hospital Mental Health while promoting safe and cost-effective prescribing for the service.

2. Terminology

SMF	The WA State Medicines Formulary (SMF) is a list of approved medications that may be initiated within the Western Australian Public Health System including medications for the treatment of mental illness. Medications listed on the SMF are grouped into 3 levels of restriction: Unrestricted; Restricted; or Non Formulary. Prescribing outside these restriction criteria is regarded as Non Formulary Prescribing.
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3. Policy

3.1. Formulary Listing

- A Statewide Medicines Formulary Submission is required for any amendments or additions to the SMF. Individuals wishing to submit a formulary application should first seek endorsement from the Drugs and

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The information provided in this document is based on its relevance to Fiona Stanley Fremantle Hospital Group (FSFHG) only. Due to differences in context, scope of practice and differences in service delivery, FSFHG does not make claim to its relevance or appropriateness for use within other organisations/sites.

Therapeutics Committee (DTC). Submissions may be completed by a Consultant Psychiatrist or DTC member. Completed formulary applications are reviewed by the WA Medication Evaluation Panel each month and if approved the relevant addition or amendment will be made to the SMF.

- Newly listed PBS medications will require a formulary application, the SMF will not be automatically updated.

3.2. Formulary Prescribing

- Prescribers must consider prescribing Formulary medications as first line (green or amber on SMF). Non-formulary or off-label indications may only be initiated where formulary items are inappropriate or ineffective
- Formulary medications may include those on the Pharmaceutical Benefits Scheme (PBS), PBS restricted and non-PBS medications. The SMF may also place additional restrictions on both PBS and non-PBS medications.
- Prescribing non-PBS medications or PBS medications outside their PBS approved indication may have cost implications for the patient. In these situations, it is recommended that the cost implications be discussed by the prescriber with the patient as this could potentially be a factor in patient compliance and the effectiveness of treatment. Documentation of this discussion must be included in patient notes.
- Medications that are on the formulary but are ineligible for PBS subsidy (non-PBS or used outside of PBS indications/restrictions) can be subsidised by a Mental Health Voucher provided by FSFHG Mental Health Outpatient Clinics, and dispensed at a [MOU Pharmacy](#).

3.3. Non Formulary Prescribing

- Medication may be termed non-formulary for a number of reasons:
 - The medication is not listed on SMF
 - The medication is listed as “Not Approved by WAMEP” on the SMF
 - The medication is listed on the SMF but is being used for an indication other than designated on the SMF or
 - The medication is being used for an approved condition however the patient does not meet all the restrictions set by SMF and/or the PBS.
- Prior to prescribing a non-formulary medication, an individual patient approval (IPA) must be sought. Applications are to be completed via the WA Health IPA System (WAIPAS). IPA applications must include the rationale for treatment (including other treatments that have been trialled), cost and evidence for treatment choice. WAIPAS applications can be completed by medical staff or a pharmacist and require endorsement from the corresponding Head of Service.

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- Once completed the IPA will be assessed by the DTC at the next weekly meeting. If urgent approval is required (ie prior to the next weekly DTC meeting) then the Medical Executive On Call must be contacted for verbal approval. See appendix 1 for details. If verbal approval is obtained this should be documented in the IPA.
- If the treatment cost exceeds \$10,000 per patient per year/course the appropriate Medical Co-Director endorsement is also required and should be included in the endorsement section of the IPA.
- Once approval has been granted, the prescriber must document in the patient's digital medical record the reason for prescribing a non-formulary or off-label medication and the point at which treatment will be reviewed. The treatment must be reviewed regularly, with a report back submitted to the DTC as requested, and treatment must be ceased if benefit is not demonstrated.

3.4. Inpatient Prescribing

- Where prescribers are considering prescribing non-formulary medications, they should involve a clinical pharmacist in the decision-making process to ensure there are no available alternatives with respect to clinical suitability and/or accessibility. When an IPA is completed, the corresponding clinical pharmacist should be listed as an interested party on the application to ensure they are aware of the outcome and can arrange prompt supply of the medication.
- Continuation of non-formulary medication is regarded as acceptable for continuation of treatment that is clinically indicated. This may require using the patient's own medication.

3.5. Outpatient Clinic Prescribing

- It is recommended that prescribing be confined to the treatment of mental illness. Medication to treat simple side effects of psychotropic medications may be prescribed where the side effect is not a medical condition requiring ongoing medical monitoring. This includes medication such as benztropine for extrapyramidal side effects and aperients for medication-induced constipation. Side effects that can be considered as medical conditions requiring ongoing medical monitoring such as metabolic syndrome must be managed by a General Practitioner or appropriate specialist.
- Prescribing for conditions other than mental health conditions requires the approval of the Head of Service via the [Non-Psychiatric Prescribing Request Form](#).
- The non-psychiatric request form must be emailed to the pharmacy department and documented in the patient's notes after approval.

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4. Compliance/Performance Monitoring

- Compliance with this policy will be evaluated biannually using data from the Best Practice Prescribing Software by the pharmacy department and reported to the Service Director.

5. Related Policy Documents

- [Statewide Medicines Formulary Policy](#)
- [WA Statewide Medicines Formulary Guidelines](#)

6. Related Standards

- NSQHS
 - Medication Safety
- NSMHS
 - Delivery of Care

7. Authorisation

EXECUTIVE SPONSOR: Medical Director Service 2					
Version	Date Issued	Compiled/Revised By	Committee/Consumer Group Consulted	Endorsed By	Revision due
1	01/2019	A/Chief Pharmacist, Fremantle Hospital	DTC Committee	Policy or Executive Committee	02/2022
2	06/2022	Chief Pharmacist, Fremantle Hospital	DTC Committee	Policy or Executive Committee	09/2025
3	08/2025	Chief Pharmacist, Fremantle Hospital	DTC Committee	Service 2 SQR Committee	08/2028

8. Appendix

8.1. Appendix 1: Urgent IPA Submissions Process

